Treatment scope · NCCP 00259 Lung monotherapy context only. The breast guide and its evidence remain separate. Discuss the exact disease setting and alternatives with the treating team.

NCCP 00259 · VERSION 6

Vinorelbine capsules · non-small-cell lung cancer

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Once-weekly capsules

How this medicine supports your treatment

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

Disrupts the machinery of cell division

Binds cell-division structures	→	Microtubules cannot function normally	→	Cell division is disrupted

Vinorelbine disrupts the tiny structures a cell needs to divide. This can stop cancer cells multiplying. Healthy blood-forming cells, gut cells and nerves can also be affected.

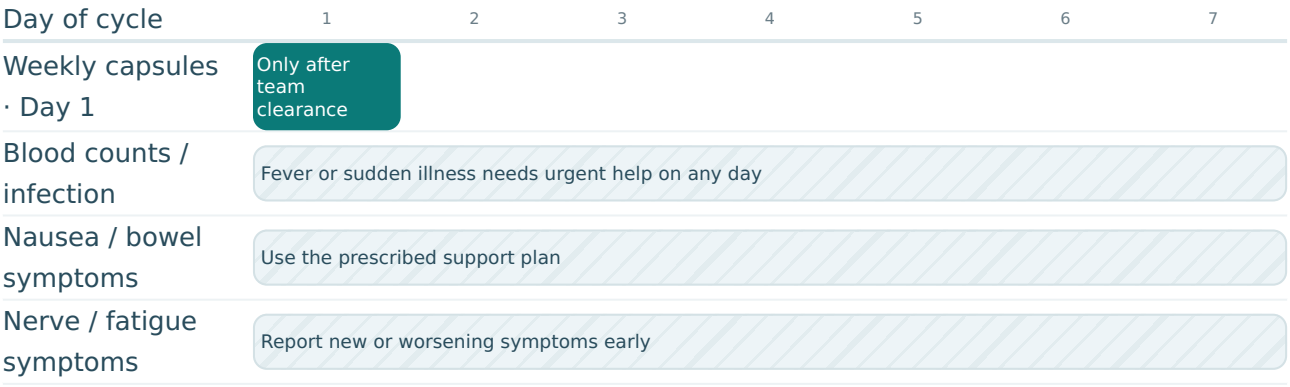
Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Vinorelbine capsules · non-small-cell lung cancer

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.



Treatment and recovery: ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Tests and decisions: Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Why these bands stay broad: This is a relative reference, not an appointment calendar. Treatment days can change after clinical review. Symptom bands do not predict when an effect will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00259 v6 · official regimen ↗

There are no guaranteed “safe days”. A day without treatment is not automatically a safe day. Use the emergency contact when needed; do not move or restart doses yourself.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver	Baseline blood and organ tests	Counts before every weekly dose	Weekly treatment review	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check	Confirm weekly capsule plan	Review nadirs after doses 1–3	Dose increases only after team review	Keep previous dose / nadir history	Personal plan; no booked dates implied

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00259 v6 · official regimen · HPRA Vinorelbine Accord soft capsules SmPC, 9 April 2025

THE WHOLE TREATMENT JOURNEY

Vinorelbine capsules · non-small-cell lung cancer: treatment and checks

Non-small-cell lung cancer · weekly oral monotherapy

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00259	Confirm lung monotherapy plan and current dose	Weekly doses 1–3; review before dose 4	Continue while helpful and tolerated	Review dose and recovery before restart	Agree continuation / next treatment
Blood / kidney / liver NCCP 00259 · SMPC	Baseline blood and organ tests	Counts before every weekly dose	Weekly treatment review	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines NCCP 00259 · SMPC	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Special schedule check NCCP 00259 · SMPC	Confirm weekly capsule plan	Review nadirs after doses 1–3	Dose increases only after team review	Keep previous dose / nadir history	Personal plan; no booked dates implied

What each check is for

1

Treatment and dose review

Before every administration

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Ask / act: The treating team confirms results, dose and suitability; this map does not authorise treatment.

Sources: NCCP 00259 · SMPC

2

Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Ask / act: Keep a written medicine plan and urgent-contact route.

Sources: NCCP 00259 · ORAL-ANTIEMETIC · NCIS

3

Response and continuation

At agreed clinical review points

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

Ask / act: Confirm the current plan with the team.

Sources: NCCP 00259 · TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?

- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

The treatment being discussed

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

Lung treatment context

NCCP 00259b: non-small-cell lung cancer. This guide covers vinorelbine monotherapy. Platinum combinations, adjuvant treatment and radiotherapy combinations need their own complete plans; this guide does not authorise them.

NCCP 00259 v6 · official regimen

Schedule and duration

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

NCCP 00259 v6 · official regimen

How treatment is given

Take with some food; swallow capsules whole with water. Do not chew, suck or dissolve. The exact total dose may require more than one capsule; pharmacy provides the strengths and number. Verify the weekly date, dose and capsule count each time. Do not take another dose if vomiting occurs within a few hours. Contact the team and record the dose taken. Do not swallow damaged capsules. Return them to pharmacy; immediately wash any contact with contents using water or saline. Missed/late dose: contact the team for the next date. Do not double or move the weekly schedule yourself. This is pragmatic medication-safety advice, not an extra NCCP catch-up schedule.

NCCP 00259 v6 · official regimen

Checks before and during treatment

Every dose depends on the reviewed treatment plan and results.

Blood and symptom checks

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

NCCP 00259 v6 · official regimen

Heart and response checks

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial LVEF/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

NCCP 00259 v6 · official regimen

Fertility, pregnancy and breastfeeding

Discuss the disease-control aim, likely benefit and uncertainty, other lung-cancer treatments or symptom-focused care, and the burden of visits and tests. Review low blood counts and infection, nausea/vomiting, fatigue, constipation/ileus, nerve symptoms, hair changes and uncommon serious lung/heart/allergic effects. Discuss once-weekly capsule safety, food, anti-sickness treatment, handling, dose escalation and no repeat dose after vomiting. Discuss fertility preservation and pregnancy planning. Do not breastfeed. Confirm the exact product contraception advice: HPRA recommends contraception during treatment and for 7 months afterwards for women, and 4 months for men.

HPRA Vinorelbine Accord soft capsules SmPC, 9 April 2025

Support and safety at home

Keep the written medicine and urgent-contact plan.

Sickness plan

Take the anti-sickness medicine exactly as prescribed before/around the weekly capsule dose; contact the team if it is not controlling symptoms.

NCCP 00259 v6 · official regimen

Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

NCCP 00259 v6 · official regimen

Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

NCCP 00259 v6 · official regimen

Weekly safety

Take capsules only once weekly after the team clears the blood results. Do not repeat a vomited dose or increase the capsules yourself.

NCCP 00259 v6 · official regimen

Urgent contact

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

NCCP 00259 v6 · official regimen

Practical support

Sickness plan

Take the anti-sickness medicine exactly as prescribed before/around the weekly capsule dose; contact the team if it is not controlling symptoms.

Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

Weekly safety

Take capsules only once weekly after the team clears the blood results. Do not repeat a vomited dose or increase the capsules yourself.

Taking tablets / attending treatment

Why this treatment may be chosen

Studies support vinorelbine as an option in selected people with advanced non-small-cell lung cancer. The strength and type of evidence differ for the IV and capsule schedules.

Historical studies do not establish superiority over current biomarker-directed or immunotherapy options. Oral and IV study results are not interchangeable.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Questions to ask

Can IV and oral vinorelbine be exchanged?

Oral 60/80 mg/m² and IV 25/30 mg/m² have pharmacokinetic equivalence references, but never convertmg-for-mg or swap routes yourself. Dose rules and schedules differ.

How long will treatment continue?

Continue weekly while helpful and tolerated; no fixed total number of weeks.

Do I need regular heart scans?

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial LVEF/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

What if I vomit, miss a dose or find damaged capsules?

Do not take another dose if vomiting occurs within a few hours. Contact the team and record the dose taken. Do not swallow damaged capsules. Return them to pharmacy; immediately wash any contact with contents using water or saline. Missed/late dose: contact the team for the next date. Do not double or move the weekly schedule yourself. This is pragmatic medication-safety advice, not an extra NCCP catch-up schedule.

How do I store the capsules?

Current Vinorelbine Accord SmPC: refrigerator 2–8°C. Confirm the dispensed brand/pack instructions; keep in original packaging and out of children’s reach. Return unused or damaged capsules to pharmacy; follow cytotoxic handling advice.

Does dose four always use more capsules?

No. The team reviews the lowest counts and side effects after the first three doses. Take only the current prescribed dose; severe or repeated low counts can prevent an increase.

Explain the plan back: Which treatment days and doses are prescribed, which blood tests must happen first, how do you take the support medicines, and which symptoms need urgent help?

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00259 v6 · official regimen
- HPRA Vinorelbine Accord soft capsules SmPC, 9 April 2025
- NCCP antiemetic guidance v6
- NCCP NCIS antiemetic medicines v8
- Gridelli et al., European Journal of Cancer 2004: weekly oral vinorelbine in older adults with advanced NSCLC
- Chouaid et al., 2025 pooled individual data from weekly oral vinorelbine arms
- 2026 correction to pooled oral vinorelbine analysis: denominator and event-estimate footnotes

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

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SACTCheck · Developed by Dr Paul O'Brien · v0.84.29 · Sources checked 2026-11-02
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Written source addresses

https://healthservice.hse.ie/documents/6363/259_Vinorelbine_Oral.pdf

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